

Neurological Consultation Report

Examination Date: 2025-12-12
Neurologist: Dr. Ahmed Jlassi, MD
Patient Name: Rami Belhaj
Patient ID: 35671289
Date of Birth: 28 April 1992
Age: 33 years
Gender: Male
Marital Status: Married
Occupation: Software Engineer

Chief Complaint

Intermittent headaches for 4 months, described as bilateral frontal pressure-like pain, lasting 26 hours, associated with photophobia and occasional nausea (no vomiting). Episodes occur 34 times per month. No aura. Pain partially relieved by paracetamol.

History of Present Illness

Headaches started gradually. No clear trigger identified, though patient reports increased screen time and work-related stress. No loss of consciousness, seizures, visual loss, weakness, numbness, or speech difficulty. No recent head trauma. Sleep pattern irregular (average 56 hours/night).

Past Medical History

- Mild asthma in childhood, resolved.
- No hypertension, diabetes, or migraine in family.

- No previous neurological events.
- Medications: Occasional paracetamol. No prophylactics.
- Allergies: None known.

Family History

Father has hypertension. No migraines, epilepsy, or stroke in first-degree relatives.

Social History

Non-smoker, no alcohol, no illicit drugs. Drinks 34 cups of coffee daily.

Neurological Examination

General: Alert, oriented, cooperative. No distress.

Mental Status:

- Level of consciousness: Fully alert.
- Orientation: To time, place, person, and situation.
- Speech: Fluent, normal comprehension and repetition.
- Memory: Immediate, recent, and remote intact (3/3 recall at 5 minutes).
- Attention & Calculation: Serial 7s performed correctly.

Cranial Nerves:

- I: Not tested.
- II: Visual acuity 20/20 bilaterally (corrected). Visual fields full to confrontation. Fundoscopy: Normal optic discs, no papilledema.
- III, IV, VI: Extraocular movements full, no nystagmus, pupils equal and reactive to light and accommodation.
- V: Facial sensation intact, jaw strength normal.
- VII: Facial symmetry preserved, no weakness.
- VIII: Hearing intact to finger rub bilaterally.
- IX, X: Palate elevates symmetrically, gag reflex present.
- XI: Sternocleidomastoid and trapezius strength 5/5.
- XII: Tongue midline, no fasciculations or atrophy.

Motor Examination:

- Bulk: Normal, no atrophy or fasciculations.
- Tone: Normal throughout.
- Strength: 5/5 in all muscle groups upper and lower limbs bilaterally.
- No pronator drift.

Sensory Examination:

- Light touch, pinprick, vibration, and proprioception intact in all four limbs.
- No extinction to double simultaneous stimulation.

Reflexes:

- Biceps, triceps, brachioradialis, knee, ankle: 2+ bilaterally symmetrical.
- Plantar responses: Flexor bilaterally (negative Babinski).
- No Hoffmann sign.

Coordination: • Finger-nose-finger and heel-shin testing smooth and accurate bilaterally.

- Rapid alternating movements normal.
- No dysmetria or intention tremor.

Gait and Station: • Normal base, stride, and arm swing.

- Tandem gait intact. Romberg negative.

Assessment

Tension-type headache, episodic, likely related to stress, irregular sleep, and excessive screen exposure. No red flags identified on history or neurological examination.

Plan

- Lifestyle modifications: Regular sleep schedule (7-8 hours), stress management, regular breaks from screens (20-20-20 rule).
- Headache diary to identify triggers.
- Trial of prophylactic amitriptyline 10 mg at night, increase to 25 mg if tolerated after 1 week.
- Continue paracetamol or ibuprofen as needed for acute attacks.
- Follow-up in 8 weeks or sooner if symptoms worsen.
- No indication for neuroimaging at this time (normal exam, no red flags).

Neurologist:

Dr. Ahmed Jlassi, MD

Consultant Neurologist

Report completed and signed on 2025-12-12